

SENATE No. 292

The Commonwealth of Massachusetts

PRESENTED BY:

John C. Velis

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act relative to growing resources to optimize the utilization of group therapeutic care.

PETITION OF:

NAME:

John C. Velis

DISTRICT/ADDRESS:

Hampden and Hampshire

SENATE No. 292

By Mr. Velis, a petition (accompanied by bill, Senate, No. 292) of John C. Velis for legislation to create coverage and regulations for group therapeutic care. Consumer Protection and Professional Licensure.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to growing resources to optimize the utilization of group therapeutic care.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 13 of the General Laws is hereby amended by adding the following
2 section:-

3 Section 110. (a) There shall be a board of certification of group therapeutic specialists,
4 called the board in this section. The board shall consist of 12 members who shall be residents of
5 the commonwealth as follows: the commissioner of the department of public health or a
6 designee, who shall serve as chair of the board; the commissioner of the department of mental
7 health or a designee; eight persons appointed by the governor; one person appointed by the
8 speaker of the house of representatives; and one person appointed by the president of the senate.
9 Of the appointed members of the board, 1 shall be a certified group therapeutic specialist,
10 provided that such person initially appointed to the board shall be eligible for board certification
11 as a group therapeutic specialist and shall obtain such certification within a time frame specified
12 in regulations adopted by the board; 1 shall represent an organization that provides group

therapeutic mental health care; 1 shall represent a community health center; 1 shall represent the National Association of Mental Illness; 1 shall represent a community provider of behavioral health care; 1 shall represent a member of a health justice community with limited access to behavioral health care; 1 shall be an individual with lived experience of utilizing or seeking to access the mental health system; and 3 shall represent the public, subject to section 9B of chapter 13, and shall be familiar with group therapeutic behavioral health care.

The term of office of each appointed member of the board shall be 3 years, except that of the initially appointed members of the board of which 3 members shall be appointed for terms of 1 year, 4 members for terms of 2 years, and 5 members for terms of 3 years, at least 1 of which is a certified group therapeutic specialist. Upon the expiration of his term of office, a member of the board shall continue to serve until the appointment of a successor. Any member of the board may be removed by the governor for neglect of duty, misconduct, malfeasance or misfeasance in office after being given a written statement of the charges against him and sufficient opportunity to be heard thereon. Upon the death, resignation or removal for cause of any member of the board, the governor shall fill the vacancy for the remainder of that member's term.

(b) The board shall meet at least quarterly or more often upon the call of the chair at such times and places as the chair shall designate. The board shall, at its first meeting and annually thereafter, elect from among its members a vice-chair and a secretary. Such officers shall serve until their successors are elected and qualified. A quorum of the board shall consist of a majority of the members. The board shall keep records and minutes as are necessary to carry out its duties and as otherwise required by law. Board members shall serve without compensation, but shall be reimbursed for actual and reasonable expenses incurred in the performance of their official

35 duties. The board may be supported with a director, administrative assistant and such other
36 professional staff and operating expenses as are required to discharge its duties.

37 (c) In addition to the powers and duties set forth in subsections (a) and (b), the board shall
38 have the following powers and duties:

39 (1) to develop and administer a program for certification of group therapeutic specialists,
40 or to determine existing certification programs which can be leveraged to this purpose, and to
41 establish education, training, experience and other qualifications for such certification,
42 application requirements and standards for practice as a certified group therapeutic specialist;

43 (2) to set standards and requirements for the establishment, board evaluation and board
44 approval or board acceptance of group therapeutic specialist education and training programs in
45 the commonwealth, the successful completion of which shall make individuals eligible to apply
46 to the board for certification as a group therapeutic specialist;

47 (3) to set standards and requirements for approval or acceptance of continuing education
48 courses and programs as the board may require for the biennial renewal of a group therapeutic
49 specialist certificate;

50 (4) to establish minimum education, training, experience and other qualifications that a
51 certified group therapeutic specialist shall possess to qualify as a trainer in any education,
52 training or continuing education program for group therapeutic specialists approved or accepted
53 by the board;

54 (5) to establish and implement procedures for receipt, review and action upon
55 applications for certification as a group therapeutic specialist and the biennial renewal thereof,

for which the board may require documentation of completion of continuing education, and the issuance of certificates of competency to practice as a group therapeutic specialist;

(6) to make, adopt, amend, repeal and enforce such rules and regulations consistent with law as it deems necessary for the protection of the public health, safety and welfare and for the proper administration and enforcement of its responsibilities;

(7) to collect reasonable fees established pursuant to section 3B of chapter 7 and to deposit such fees into the Quality in Health Professions Trust Fund pursuant to section 35X of chapter 10 to support board operations and administration;

(8) to identify and adopt a certification examination or other means to assess group therapeutic specialist competency in connection with board certification, should the board determine that such examination would enhance the advancement of the practice and profession of group therapeutic specialists;

(9) to certify as a group therapeutic specialist an applicant who has been duly certified as a group therapeutic specialist under the laws of another state, territory or commonwealth of the United States or the District of Columbia where the requirements for certification are, in the opinion of the board, equivalent to those in the commonwealth for group therapeutic specialists, or who maintains current certification from a nationally-recognized certifying body as a group therapeutic specialist or the equivalent, and such applicant submits documentation satisfactory to the board of such certification and the requirements therefore;

(10) to establish grounds for complaints related to the practice of group therapeutic specialists and to establish and implement procedures for the review, investigation and resolution of such complaints;

(11) to establish the disciplinary actions available to the board in connection with complaint resolution, which may include a fine, reprimand, probation or censure or suspension, revocation or denial of a certificate issued by the board or a combination of the foregoing and to discipline certificate holders in accordance with procedures established by the board that shall conform with chapter 30A and 801 CMR 1.01 et seq.; and

(12) to perform such other functions and duties as may be required to carry out this section.

SECTION 2. (a) The division of medical assistance and its contracted health insurers, health plans, health maintenance organizations, behavioral health management firms and third-party administrators under contract to a Medicaid managed care organization, accountable care organization or primary care clinician plan shall provide coverage of group therapeutic care services provided by certified group therapeutic specialists. In determining the scope of group therapeutic care services, the division shall consult with the department of public health and the department of mental health.

(b) The division shall promulgate regulations to establish a payment rate for group therapeutic care services provided by a certified group therapeutic specialist.

(c) The division shall establish a pilot program to promote and evaluate the use of group therapeutic behavioral health care for MassHealth members. The pilot program and evaluation shall include, but not be limited to, the following:

(1) identifying practice locations of certified group therapeutic specialists and support the establishment of additional certified group therapeutic specialist practices;

(2) establishing a process for encouraging the referral of MassHealth members to certified group therapeutic specialists;

(3) informing providers and members of the availability and benefits of and the encourage them to use such care, including through a public awareness campaign; and

(4) tracking data on the use of group therapeutic behavioral health care, including demographic data and outcomes, to permit evaluation of the effectiveness of the pilot program, particularly for people with limited access to behavioral health care.

The division may determine the geographic scope of the pilot program; provided, that the program shall at a minimum focus on regions with limited access to behavioral health care services or at risk due to limited access to social determinants of health.

All findings and data from the project shall be made available to the health policy commission, the committees on ways and means and the joint committees on health care financing and mental health, substance use and recovery.

The division shall begin the project no later than 1 year after the effective date of this act and shall complete its evaluation of the project no later than 3 years after the effective date of this act.

SECTION 3. (a) The health policy commission shall conduct a comprehensive study and issue a report evaluating the systemic outcomes, demand, and potential cost-savings of integrating group therapeutic behavioral care into the commonwealth's behavioral health care system. The report shall include recommendations on short and long-term steps the

119 commonwealth can take to improve access to effective, high-quality behavioral health care
120 through group therapeutic care.

121 The study shall include, but not be limited to, analysis of the following topics:

122 (1) current utilization and prevalence of group behavioral health care in the
123 commonwealth;

124 (2) a cost-benefit analysis of expanding access to group therapeutic care;.

125 (3) identification of barriers to accessing traditional individual therapy that could be
126 mitigated through group care, including racial and socioeconomic disparities, cultural
127 competence, financial burdens and availability of providers;

128 (4) comparative analysis of the efficiency and effectiveness of outcomes in group
129 therapeutic care as compared to individual therapy; and

130 (5) assessment of the use of culturally competent techniques and minority practitioners in
131 group therapeutic care relative to individualized therapy.

132 (b) The commission shall consult with the department of mental health, the division of
133 medical assistance and other agencies of the commonwealth, and with individuals, organizations
134 and providers with expertise in group therapeutic behavioral care. The commission shall hold a
135 public comment session to receive information and recommendations from members of the
136 public.

137 (c) The report shall be issued no later than December 1, 2026, and be made available
138 electronically on the commission's website, and shall be filed with the secretary of
139 administration and finance, the secretary of health and human services, the clerks of the house of

140 representatives and the senate, the house and senate committees on ways and means, the joint
141 committee on mental health and substance use and recovery and the joint committee on health
142 care financing.